

Barcode No	89781002	Lab No	16952412180002
Patient Name	Mrs.RAMEET KOUR	Reg Date	18/Dec/2024 10:05AM
Age/Sex	29 YRS/Female	Sample Coll. Date	18/Dec/2024 09:08 AM
Referred By	DR. ROBINA MIRZA	Sample Rec.Date	18/Dec/2024 10:21 AM
		Report Date	18/Dec/2024 04:23PM

IMMUNOLOGY SPE

Test Name With Methodology	Result	Unit	Biological Ref.Interval
Dual Marker			
PAPP-A (Pregnancy Associated Plasma Protein A) Chemiluminescent immunoassay (CLIA)	593.7	ng/ml	
Beta Hcg Titer Chemiluminescent immunoassay (CLIA)	92189	mIU/mL	

Statistical evaluation of **Dual Marker** has been done using **Benetech PRA software**.

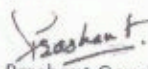
The test is valid between 10 Weeks to 13 weeks & 6 days of gestation.

Dual or Double marker test in first trimester, identifies pregnancies that have a high risk of babies having the following chromosomal abnormalities:

1. Trisomy 21 (Down syndrome)
2. Combined Trisomy 13 (Patau syndrome)/ Trisomy 18 (Edwards syndrome)

This Screening test is NOT a diagnostic test (it cannot tell whether the baby has any of the above conditions); it only identifies the risk of the baby being affected. A confirmatory test may be needed when the screening test is positive (high risk)

The interpretive unit is MoM (Multiples of Median) which takes into account variables such as gestational age (ultrasound), maternal weight, race, insulin dependent Diabetes, multiple gestation, IVF (Date of Birth of Donor, if applicable), smoking & previous history of Down syndrome. Accurate availability of this data for Risk Calculation is critical.


 Dr. Prashant Goyal (DCP)
 (Director & Chief Pathologist)
 Reg. No. DMC-53016

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 Patient Name **Mrs.RAMEET KOUR**
 Age/Sex **29 YRS/Female**
 Referred By **DR. ROBINA MIRZA**

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 Report Date **18/Dec/2024 11:12AM**

Test Name With Methodology	Result	Unit	Biological Ref.Interval
HAEMATOLOGY			
Haemoglobin HPLC			
Hb F(Foetal Hb) Level	0.6	%	< 2.0
Peak 2	3.6	%	
Peak 3	4.4	%	<10
Hb A0	81.7	%	
HbA2	2.9	%	1.9 - 3.5
Hb D (Punjab)	0.0	%	<0.02
Hb Sickie	0.0	%	<0.02
Hb C	0.0	%	<0.02
Hb E	0.0	%	<0.02
Opinion	Normal Pattern		
Advise	Please Correlate Clinically		

Comment:

Haemoglobinopathy is a kind of genetic defect that results in abnormal structure of one of the globin chains of the hemoglobin molecule. Hemoglobin variants are abnormal forms of hemoglobin. All results have to be correlated with age and history of blood transfusion. If there is history of blood transfusion in last 3 months, repeat testing after 3 months from last date of transfusion is recommended. **In case of hemoglobinopathy, parents or family studies, DNA Analysis and Genetic counseling is advised.**

- ~ **P3 window-** Above 10% is often indicative of either denatured forms of hemoglobin (degenerated sample) or may suggest a possibility of abnormal hemoglobin variant. Hence, repeat analysis with fresh sample or DNA studies is advised.
 - ~ **P2 window-** Above 10% is indicative of either glycated hemoglobin requiring correlation with diabetic status or may suggest a possibility of abnormal hemoglobin variant requiring further DNA studies for confirmation.
 - ~ Screening of the spouse are strongly recommended, if pregnant woman have hemoglobinopathy.
 - ~ This test detects Beta thalassemia and hemoglobinopathies. DNA analysis is recommended to rule out alpha thalassemia and silent carriers.
 - ~ Mild to moderate increase in fetal hemoglobin (HbF) can be seen in some acquired conditions like Pregnancy, Megaloblastic anemia, Thyrotoxicosis, Hypoxic conditions, Chronic kidney disease, Recovering marrow, MDS, Aplastic anemia, PNH, Medications (Hydroxyurea, Erythropoietin, Butyrates) etc.
 - ~ Iron deficiency Anemia may be associated with low HbA2 result and may mask thalassemia condition also.
 - ~ **Elevated levels of HbA2** are seen in Beta-Thalassemia, and also in other conditions like Sickie cell Alpha-Thalassemia, Megaloblastic Anemia, Unstable Hemoglobinopathies, Zidovudine- treated HIV patients, Congenital Dyserythropoietic Anemia (CDA) type 1 and Hyperthyroidism.
- *Results relate only to the sample, as received.


 Dr. Urvashi, DNB, Path
 (Consultant Pathologist)
 Reg. No. DMC-84904

 Dr. Prashant Goyal (DCP)
 (Director & Chief Pathologist)
 Reg. No. DMC-53016

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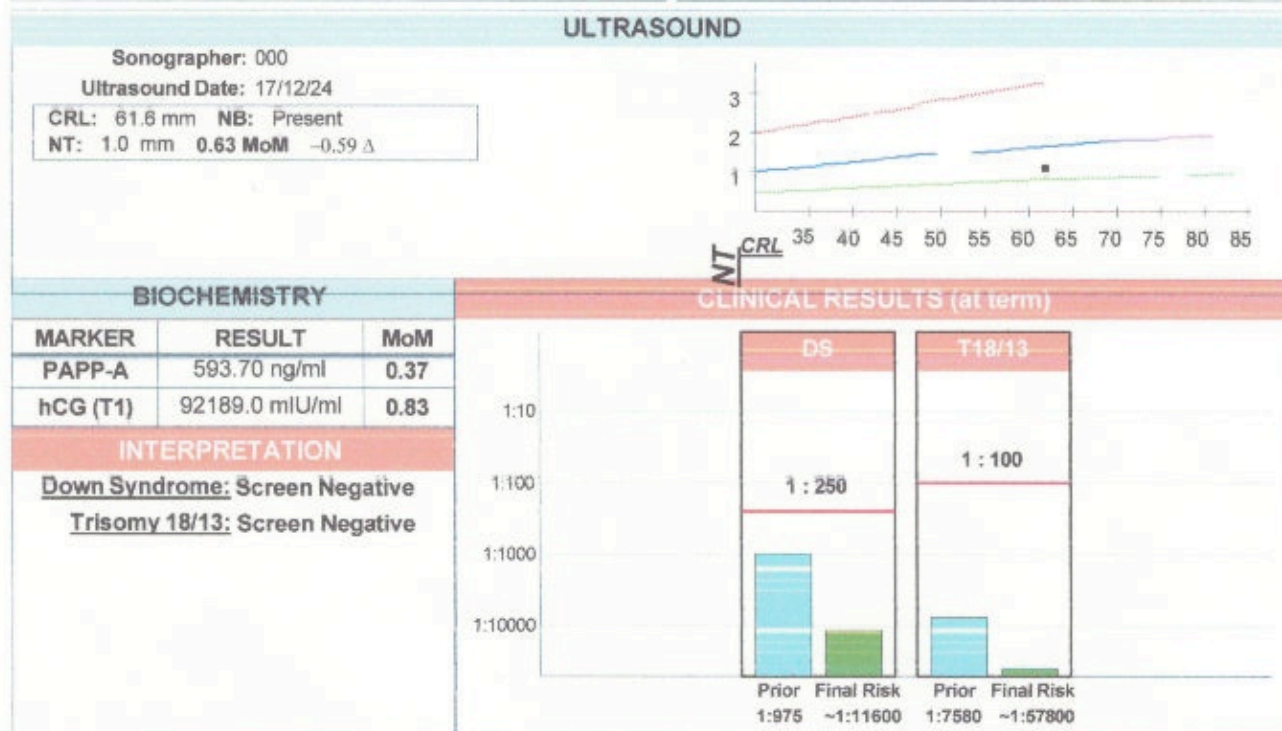
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DUAL MARKER

Patient Name: Mrs. RAMEET KOUR Race: INDIAN Physician:	Code: 89781002 DOB: 30/07/95 Reported: 18/12/24
CLINICAL INFORMATION Estimation Method: by U/S (12 wks 4 days on 17/12/24) Age at Term: 29.9 years EDD: 27/06/25 Gestation: Singleton Maternal History: IDDM(N), SMOKER(U), IVF(U) Gestational Age: 12 weeks 5 days Referring Lab #: 89781002 Specimen Code: 89781002 Specimen Date: 18/12/24 Received Date: 18/12/24 Weight: 65.0 kg Screening Status: Initial sample Para / Gravida: 0 / 0	REMARKS <u>Down Syndrome</u> The risk of Down syndrome is LESS than the screening cut-off. <u>Trisomy 18/13</u> The risk of trisomy 18 is less than the screening cut-off.



The calculated risk by Benetech depends on the accuracy of the information Provided by the referring physician. Please note that calculation are statistical approaches and have no diagnostic value.

Report Validated by DR
PRASHANT GOYAL

Chromatogram Report

Tosoh HLC723G11 V3.07 1

2024-12-18 10:55:20

ID 89781002

Sample No. 2024121810380057 SL 0001 - 02

Patient ID

Name

Comment

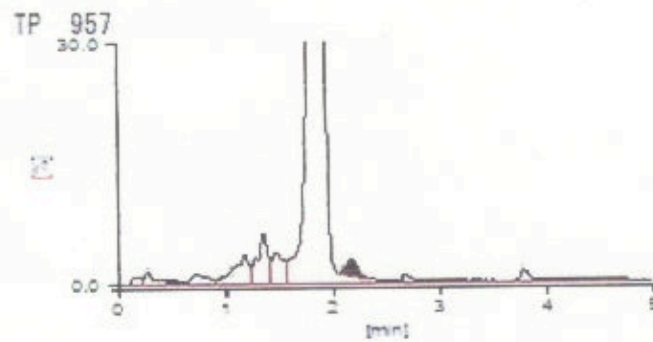
CALIB F $Y = 1.3419X + 0.0998$

A2 $Y = 1.3909X + 0.9070$

Name	%	Time	Area
F	0.6	0.50	5.00
A0	81.7	1.88	1076.72
A2	2.9	2.17	20.69
E+			
D+			
S+			
C+			

Total Area 1318.52

HbF 0.6 % HbA2 2.9 %



[Unknown Peak]

Name	%	Time	Area
P00	0.5	0.15	7.05
P01	0.9	0.27	12.11
P02	1.4	0.73	17.81
P03	3.6	1.18	47.22
P04	4.4	1.36	57.81
P05	3.0	1.47	39.54
P07	1.3	3.81	16.70